

Public Health Emergency Operations Center

"COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°065/MVB_18/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (46)	• Haut-Uélé (4/13): Wamba, Boma Mangbetu, Pawa and Isiro • Ituri (27/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara and Mahagi. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Makiso-Kisangani, Lubunga and Mangobo
Reporting date	July 18, 2026
Publication date	July 19, 2026

KEY INDICATOR S — MUL CURRENT AS OF JULY 18, 2 0 26

CONFIRMED CASES — 5 PROVINCES 2,344* +83 new cases (Ituri 67 · North Kivu 16)	CONFIRMED DEATHS · LEATHITIS 930* · (39.7%) +40 deaths (Ituri 31 · North Kivu 9)	SUSPECTED CASES OF THE DAY 192 Ituri 128 · N-Kivu 57 · Tshopo 3 · Haut-Uele 4
PATIENTS IN ISOLATION 724 Ituri 539 · N-Kivu 181- Tshopo 4	CURED — CUMULATIVE 466 +22 recovered (Ituri 21 – North Kivu 1)	NATIONAL CONTACT TRACKING 85.8% 10517/ 12251 views · Ituri 84.9% N-Kivu 88.4% - Tshopo 100.0%

*Harmonization of data on confirmed cases and deaths between the province of Haut-Uele and the Nia-Nia health zone in Ituri.

1. Epidemiological Highlights

- **Epidemiological situation** : No new affected health zones have been reported; the cumulative total remains at 46 affected health zones out of 140 in the five provinces, of which 45 remain in active transmission.
- **Case trends** : In the last 24 hours, 83 new confirmed cases, 40 confirmed deaths and 22 Recoveries have been recorded.
 - o **Ituri**: 67 new confirmed cases were reported, mainly in Nizi (30), Bunia (15) and Rwampara (10). The province recorded 31 deaths, including 9 in Ebola Treatment Centers/Treatment Centers.
 - o **North Kivu**: 16 new confirmed cases were reported (Katwa 6, Musienene 4, Beni 3, Butembo 2 and Kyondo 1) as well as 9 deaths, including one at the Katwa Ebola Treatment Center. The provincial total reaches 230 confirmed cases and 139 deaths, representing a case fatality rate of 60.4%.
 - o **Haut-Uélé** : No new cases or deaths were recorded. Following the harmonization of data with the Nia-Nia health zone (Ituri), the provincial total has been revised to 16 confirmed cases and 10 deaths.
 - o **Tshopo and South Kivu** : No new confirmed cases or deaths have been reported. The cumulative totals remain at 5 cases, 4 deaths and 1 recovery in Tshopo, and 3 cases and 1 death in South Kivu, where the Miti-Murhesa health zone has gone 52 days without a new confirmed case.

- **Analysis:** Ituri remains the epicenter of the epidemic, accounting for 89.1% of cumulative cases and 80.7% of new confirmations. Contact tracing performance remains insufficient and requires strengthened surveillance activities.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 46 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response interventions, and limit the spread of the epidemic.

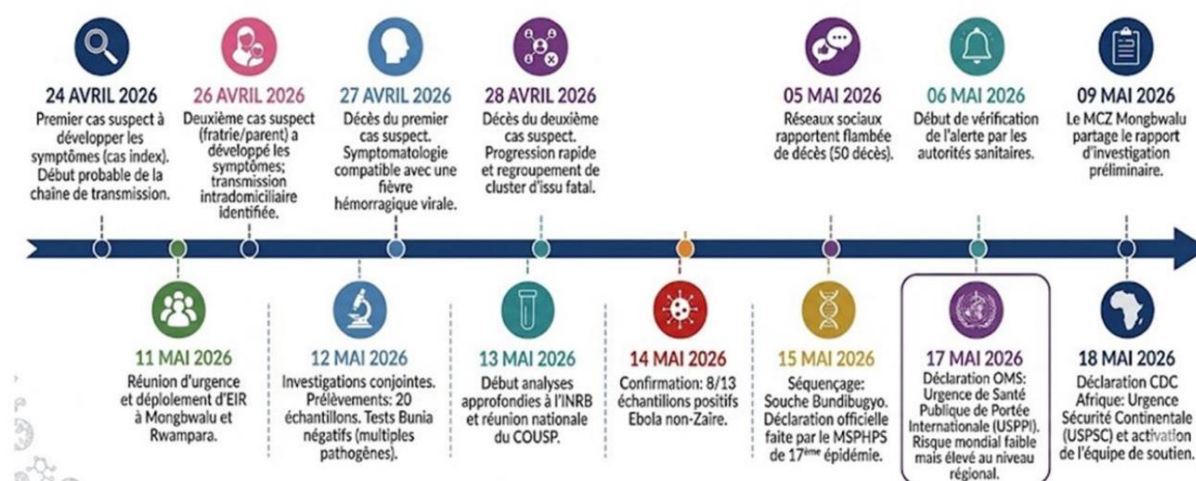


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Case fatality rate, affected health zones, new cases	(24 h)
Ituri	2090	776	37.1% 27 / 36 (75.0%)	67
North Kivu	230	139	60.4% 11/34 (32.4%)	16
South Kivu	3	1	33.3% 1/34 (2.9%)	
Haut-Uélé	16	10	62.5% 4/13 (30.8%)	
Tshopo	5	4	80.0% 3/23 (13.0%)	0
Total	2,344	930	39.7% 46 out of 140	83

The Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) epidemic, accounting for 89.1% of confirmed cases ($n = 2,090$) and 83.2% of deaths ($n = 776$), representing a case fatality rate of 37.1%. The health zones of Bunia (621 cases), Rwampara (440), Mongbwalu (360), Nizi (198), and Nyankunde (99) remain the most affected. The 83 new confirmed cases of July 18 are distributed in the health zones of Nizi (30), Bunia (15), Rwampara (10), Katwa (6), Mongbwalu (5), Musienene (4), Beni (3), Butembo (2), Fataki (2), Nia-Nia (2), Mangala (2), Kyondo (1) and Tchomia (1).

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community death res (Swab+).	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	2090	776	37.1%	67	22	9	31
Bunia	621	202	32.5%	15	4	1	5
Rwampara	440	124	28.2%	10	6	2	8
Mongbwalu	360	199	55.3%	5	1	2	3
Nizi	198	87	43.9%	30	8	0	8
Nyankunde	99	28	28.3%				
Lita	77	27	35.1%				
Mangala	58	35	60.3%	2	2	0	2
Nia-Nia	49	15	30.6%	2	1	2	3
Bamboo	36	12	33.3%				
Komanda	27	10	37.0%				
Tchomia	22	5	22.7%	1	0	0	0
Unidentified	17	0	0.0%				
Fataki	11	7	63.6%	2	0	1	1
Kilo	11	3	27.3%				
Mandima	9	4	44.4%				
Mambasa	8	6	75.0%	0	0	1	1
Damascus	7	0	0.0%				
Logo	7	1	14.3%				
Ariwara	6	1	16.7%				
Aungba	6	2	33.3%				
Drodro	5	3	60.0%				
Rimba	5	0	0.0%				
Aru	4	1	25.0%				
Kambala	2	2	100.0%				
Lolwa	2	1	50.0%				
Boga	1	1	100.0%				
Gety	1	0	0.0%				
Mahagi	1	0	0.0%				
North Kivu	230	139	60.4%	16	8	1	9
Katwa	98	71	72.4%	6	3	1	4
Butembo	57	29	50.9%	2	2	0	2
Blessed	37	25	67.6%	3	1	0	1
Musienene	18	5	27.8%	4	2	0	2
Kyondo	9	4	44.4%	1			
Kalunguta	2	1	50.0%				
Masereka	3	0	0.0%				
Oicha	3	3	100.0%				
Goma	1	0	0.0%				
Vuhovi	1	1	100.0%				
Mabalako	1	0	0.0%				
South Kivu	3	1	33.3%	—			—

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community death res (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Miti-Murhesa	3	1	33.3%				
Haut-Uélé	16	10	62.2%	—			—
Wamba	6	3	50.0%				
Pawa	3	3	100.0%				
Boma Mangbetu	4	2	50.0%				
Isiro	3	2	66.7%				
Tshopo	5	4	80.0%	—			—
Makiso-Kisangani	3	3	100.0%				
Mangobo	1	1	100.0%				
Lubunga	1	0	0.0%				
Total	2,344	930	39.7%	83	30	10	40

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi and Nyankunde, with an extension towards North Kivu (Katwa, Butembo, Beni), Haut-Uélé (Wamba) and Tshopo.

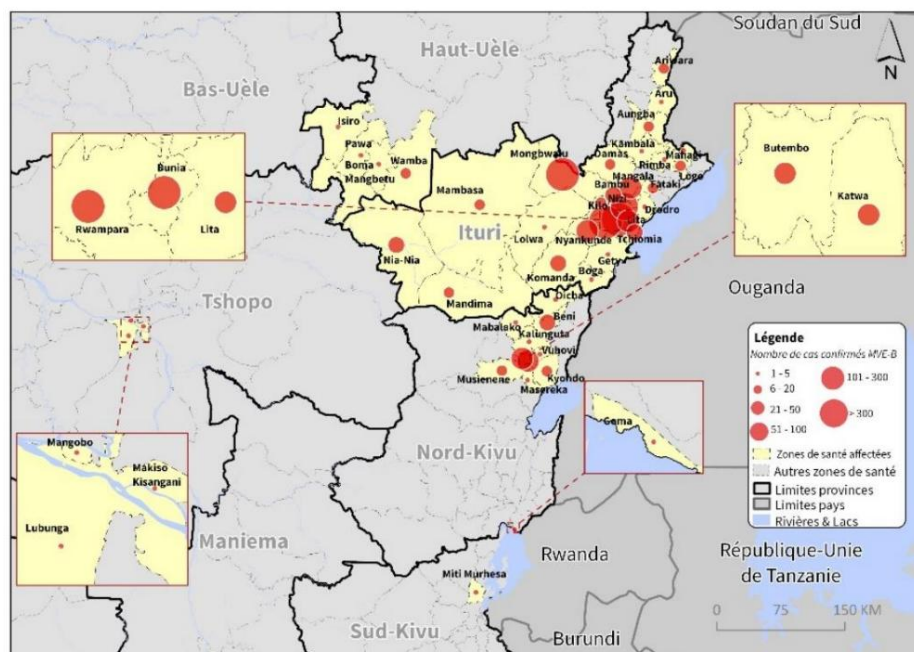


FIGURE 2 — HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES (AS OF 07/18/2026)

The recent appearance of new hotspots in Haut-Uélé confirms a progressive expansion of the affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter level and preparatory activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained community transmission, with a continuous increase in confirmed cases during epidemiological weeks S25 to S28, each recording more than 300 cases.

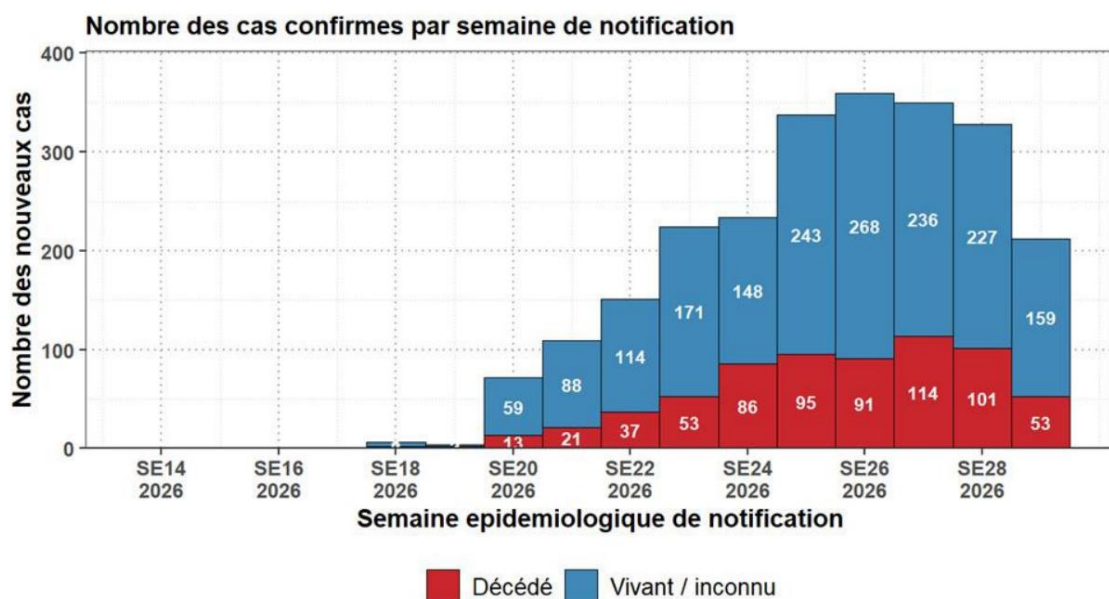


FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

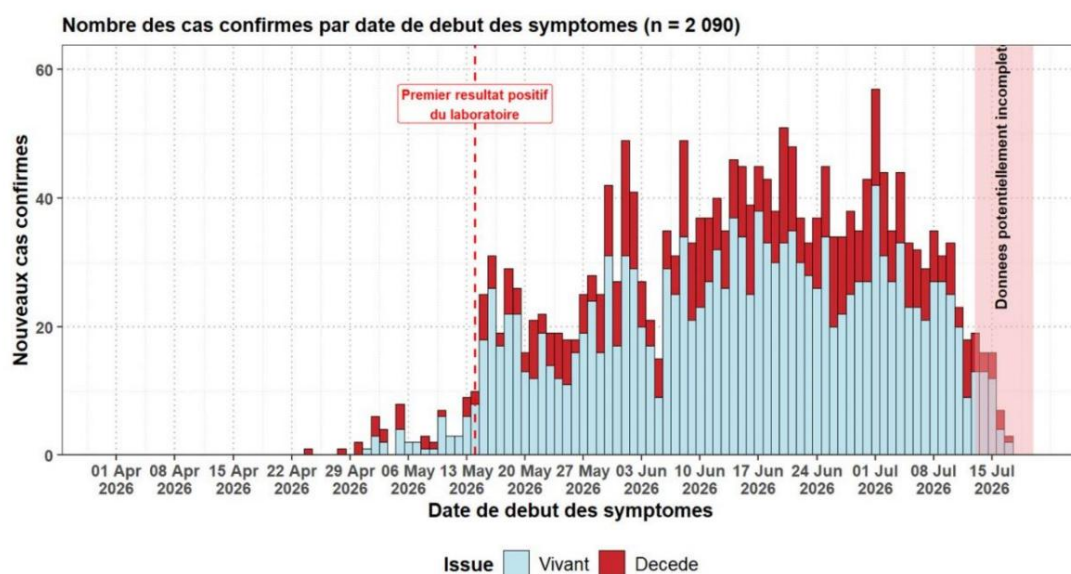


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults** (18–50 years). Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

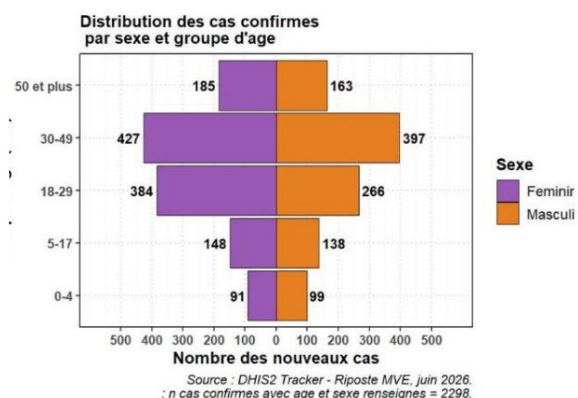
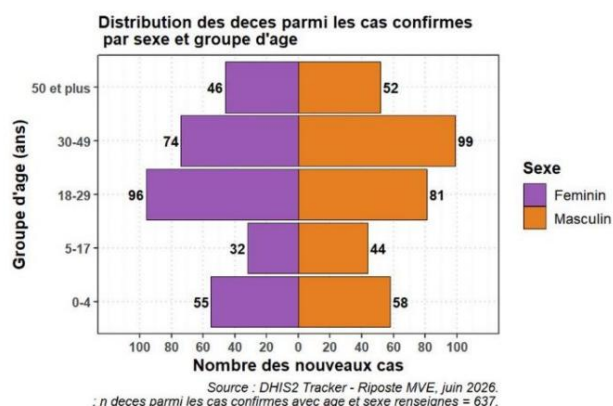


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG THE CONFIRMED CASES)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS AS OF 18/07/2026

Indicator	Ituri North Kivu		Haut-Uélé	Tshopo	Total
Report (D-1)	3	0	4	1	8
New alerts received	336	525	0	3	864
Total alerts for the day	339	525	4	4	872
Alerts investigated	293	525	4	3	825
Investigation rate (24 h)	86.4%	100.0%		75.0%	94.6%
Alerts confirmed — live	72	33	4	3	112
Alerts confirmed — deceased (comm.)	56	24	0	0	80
Total suspected cases for today	128	57	4	3	192

In total, 872 alerts (864 of which were new) were received, with 825 investigated within 24 hours (94.6%) and 192 confirmed (112 active cases and 80 community deaths). This improvement reflects a reduction in investigation delays and the consolidation of data from the affected provinces.

3.2 Laboratory: tests and positivity

• **Ituri** : 167 samples documented in the collection network, of which 67 were positive (45 live and 22 dead), i.e. a apparent positivity of 38.1%.

• **North Kivu** : 127 samples received and analyzed, of which 16 were positive, representing a positivity rate of 12.6%.

• **Haut-Uele** : 53 samples taken are currently being analyzed.

• **Tshopo** : 6 samples taken are currently being analyzed.

- Deployment of laboratory inputs, equipment and diagnostic kits in the Mahagi health zone for the installation of the MVE-B laboratory.

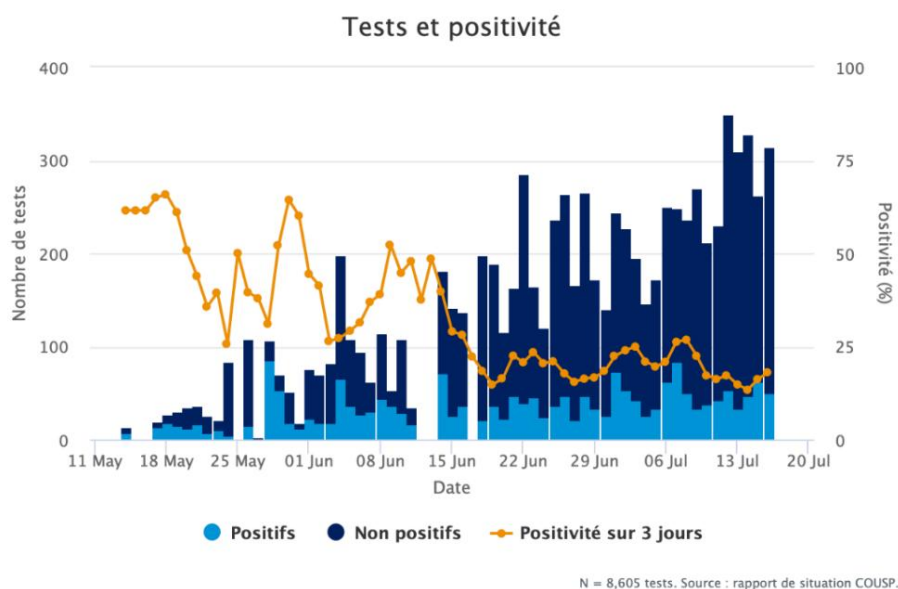


FIGURE 7 — TESTS AND POSITIVITY (COUSP SITUATION REPORT)

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	9285	7886	84.9%
North Kivu	2888	2553	88.4%
Tshopo	78	78	100.0%
Haut-Uélé	ND	ND	ND
South Kivu (end of monitoring)	0	0	N / A
Total	12,251	10,517	85.8%

Contact tracing remains insufficient at the national level (85.8%), below the operational threshold of 95%. Ituri only managed to follow up with 9,285 out of 7,886 contacts (84.9%). North Kivu maintains a high performance with 2,888 out of 2,553 contacts seen (88.4%). Tshopo achieved the best performance with a 100% follow-up rate. Contact tracing is now closed in South Kivu.

3.4 Entry points and control points (PoE/PoC)

As part of capacity building for the response, the PoE/PoC pillar received an ambulance provided by the SGI; The PoE/PoC pillar benefited from the provision of communication tools by FHI360 to strengthen response activities.

3.4.1 — Crossing point alerts (24 h)

Ituri — 3 confirmed alerts

Six live alerts were detected at the Dele, Mudzibala, Makoko 2 and Nzebi border crossings. All were validated; Five were referred to a CT/CTE. Two cases of resistance or refusal to transfer required the involvement of community monitoring and communication teams.

North Kivu — 2 alerts (1 body)

Two alerts notified, one alive at PoC Kahongo and a lifeless body intercepted at PoC Mukulia.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC), AS OF 18/07/2026

Indicators	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (15/15)	100% (75/75)
Number of people who have switched to PoE/PoC	86463	62822	149,285
% of travelers screened	92.3%	97.0%	94.3%
% of travelers who washed their hands	91.8%	97.0%	94.0%
% of travelers made aware	98.7%	97.0%	98.0%
Number of alerts notified	3	2	5
Number of validated live alerts	3	1	4
Number of bodies recovered today	0	1	1
Number of problematic contacts intercepted today; % of	0	0	0
validated live alerts referred to CT/CTE; % of deceased	66.7%	50%	60.0%
bodies swabbed	N / A	50%	50%
Number of positive results of referred suspected cases	0	0	0

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI), AS OF 18/07/2026

Indicator		Ituri	North Kivu Haut-Uele Tshopo Ensemble			
Number of beds		706	141	ND	10	857
Patients in bed (Day -1)		540	166	ND	1	707
Total admissions (24 hours)		71	47	ND	3	121
Total admissions		611	213	ND	4	828
Outings	deceased	13	1	ND	0	14
	No case	33	30	ND	0	63
	Healed	21	1	ND	0	22
	Escapees	4	0	ND	0	4
	Transferred to the HGR	1	0	ND	0	1
Total outflows (24 h)		72	32	ND	0	104
Patients in isolation (end of day)		539	181	ND	4	724
including those confirmed		257	15	ND	0	272
including suspects		282	166	ND	4	452
Occupancy rate		76.3%	123.1%	ND	40%	84.5%

In total, 724 patients were in isolation, including 272 confirmed cases and 452 suspected cases, for an overall occupancy rate of 84.5% (724 patients for 857 beds), for 828 admissions and 104 discharges (22 recovered, 14 deaths, 63 non-cases, 4 escaped and 1 transferred). Ituri bears the brunt of the burden with 539 patients in isolation (257 confirmed, 282 suspected) out of 716 beds, representing an occupancy rate of 76.3%. In North Kivu, 181 patients are in isolation (15 confirmed, 166 suspected) for an occupancy rate of 123.1% (141 beds). In Tshopo, 4 patients in isolation (0 confirmed and 4 suspected) out of 10 beds, i.e. 40% occupancy.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY INDICATORS SMSPS, JULY 18, 2026

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	26 (83.9%)	0
Confirmed cases are being monitored.	112 (67.5%)	9 (100%)
New suspected cases	33 (67.3%)	11 (100%)
Suspected cases are being monitored	105 (63.6%)	17 (100%)
Laboratory results announced (including positive ones)	53	8
Children separated — nursery / community	24	2
CTE support staff	0	0
Frontline Personnel (PPL)	0	0
Members of households and communities	175	0
EDS supported (SMSPS)	40	0

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD (CUMULATIVE AS OF JULY 18, 2026, ITURI)

Health Zone	Confirmed cases	Healed	In hospitalization	Death
Bunia	35	17	4	14
Rwampara	32	23	2	7
Mongbwalu	16	5	3	8
Nyankunde	14	11	1	2
Nia-Nia	10	0	10	0
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Lita	2	1	0	1
Total — case fatality rate 30.3%	119	61	22	36

PPL = healthcare personnel and frontline providers.

7 Pillar-Based Response Activities

COORDINATION Meeting held to coordinate monitoring of response activities in the 5 affected provinces.	PCI / WASH ÿ Ituri: 13 scorecard evaluations completed: CS SALAMA 51.6%, CI Abedi Djaka 32%, CH Amani 62%, CH Shalom 50%, Simbilibo 71%; Number of PPLs trained (100/15116) in the Lita health zone; ESS follow-ups and support of 54/364 carried out at CS SALAMA, CS BINASE, CLINIQUE ABEDI DJAKA and PS JULES. ÿ North Kivu : Completion of training for 17 IPC (Infection Prevention and Control) contractors in the Mabalako Health Zone with IRC support; Completion of training for 35 supervisors and focal points, including 15 from the Musienene Health Zone and 20 from the Katwa Health Zone, with WHO support; Decontamination of 4 health facilities (HCFs) for case stays, including 1 in Kyondo, 1 in Katwa, 1 in Beni, and 1 in Butembo; Provision of incomplete IPC kits to 14 HCFs in the Musienene Health Zone; IPC scorecard evaluation in 6 HCFs, including 1 in Musienene and 5 in Beni, all with an average score of less than 50%; Monitoring and support of 72 HCFs, including 5 in Katwa, 38 in Beni, 2 in Kyondo, and 1 in Musienene and 26 in Oicha; Briefing of 275 ESS providers during formative supervisions including; 103 in Beni, 51 in Butembo, 9 in Katwa, 14 in Kyondo; 1 in Musienene and 97 in Oicha; Securing and EDS for 16 bodies including 6 in Beni, 3 in Butembo; 6 in Katwa and 1 for Oicha; Community deaths that were the subject of SWAB: 28 including 10 in Beni, 3 in Butembo, 11 in Katwa and 4 for Oicha; Risk assessment of personnel exposure: 6 in Butembo.
Dignified and secure burials (DSM) ÿ Ituri: 52 EDS carried out out of 67, including 36 swabs, harmonization of SOPs jointly with the MSPHPS, Red Cross, FHI 360 and WHO. ÿ North Kivu: Bodies swabed and secured awaiting 12, including 24 EDS carried out	HOLISTIC CARE (HCC) ÿ Ituri: 22 recoveries and 71 new admissions, including 9 confirmed deaths, during the day of July 19: 988 meals were served to patients, including 134 to suspected cases, 161 to confirmed cases, 651 PPL and 42 to accompanying persons. Monitoring of extension and deconstruction work of CTEs; Monitoring of reporting of linear lists of all patients and recoveries; Visit to Bambu with partner GBGM for the presentation of the CTE construction project to the community; Briefing of Data from the Bambu Health Zone on the entry of PECH data in the DHIS2 tracker. ÿ North Kivu: 166 patients hospitalized. ÿ Tshopo: Compassionate treatment of 2 contacts at the FEDI CS of the Mangobo ZS with the assistance of the INRB team; 1 suspected case admitted to the HC Kisangani CTE (negative result); 2 new suspected cases: HCKis (negative result) and HGR.

COMMUNICATION (RCCE / CREC) ÿ Ituri: 1,320 people were reached during mass awareness campaigns conducted by community actors in the Health Zone (AUNGBA, NIANIA); 1 advocacy meeting was held with community leaders to solicit their involvement and support in managing resistance and acceptance of response interventions; Monitoring mission of routine activities in the BUNIA Health Zone; Downloading of some indicators on DHIS2; Development of some SOPs within the framework of continuity of care; North Kivu: Briefing of 16 journalists from Oicha, Kalunguta and Mabalako on the role of the media and the production of radio formats in communication on Ebola prevention in the Beni Health Zone; Communication of the message on prevention measures against Ebola virus disease to the faithful of the Catholic church, during a morning mass in the Mabuku sector, in the Mabuku AS.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ Ituri: Development of a synthesis of equipment needs for health zones; Mission to monitor routine activities in the Bunia health zones; Development of some SOPs within the framework of continuity of care.
RESEARCH, INNOVATION & VACCINATION ÿ Ituri: Field visit to test data collection tools with community stakeholders (CAP support); Studies on community resistance and perceptions: Dissemination of results process (WHO and UNICEF support); Clinical trials Partners (Vaccine): In progress; Anthropological study on the index case BDVD: In progress (WHO support); EBO-PEP clinical trials: In progress (ALIMA support); IPC challenge: Protocol currently being validated (Hygiene and Safety Department) Public Health	LOGISTICS ÿ Ituri: Delivery of PCI, PEC, Surveillance, Lab and CREC Kits: 2440 Kg; Delivery of Communication tools (STARLINK mini): 1 package, weighing 3 Kg; Delivery of PCI, PEC, and Surveillance Kits to the CTE HGR Bunia, LITA, CME Bunia: 16 packages weighing 210 Kg; Delivery of Communication tools (STARLINK standard), PCI kits to the WHO Office: 10 packages weighing 58 Kg; Receipt of PCI inputs, WHO support; Receipt of a Cargo of dignity Kits by WEL HUNGER HILFE (WHH). ÿ Tshopo : Deployment of communication tools; Water supply at PK 23; Storage of medicines at the HUB between -25°C and 15°C; Transport of the suspect to the Cinquantenaire CTE.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) As part of the strengthening of mechanisms for the prevention of sexual exploitation and abuse (PSEA), a briefing was organized for the response teams deployed in the provinces of Tshopo and Haut-Uélé. Nineteen experts, including two women, were trained and signed the Code of Ethics and Good Conduct, reaffirming their commitment to respecting institutional standards. Awareness-raising activities were conducted in the Saio Market in the Bunia Health Zone, CNCA Health Area, targeting women and shopkeepers on the prevention of healthcare-associated infections (HAIs), the alert number, and the referral process. A total of 15 people were reached, including 4 women, 4 men, 3 girls, and 4 boys.	

8 Main challenges, impacts and actions required

Challenge No.				Impact	Actions required
1	Reluctance to undergo post-mortem sampling (EDS/swabs), community resistance, and insecurity aimed at the EDS teams (attack at the bridge) Muchanga, paralysis of activities in Nyankunde)			Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)			Interruption of response activities, including dignified and safe burials	Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT: occupancy of 123.1% in North Kivu (bed capacity: 141) and 76.3% in Ituri (539 patients for 706 beds);			Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.
4	Low contact tracing performance in Ituri (84.9%); North-Kivu 88.4%			Non-transmission chains interrupted	Train, deploy and motivate community liaison officers (RECOs); strengthen supervision; investigate the observed drop in Ituri
5	Insufficient number of ambulances and pre-positioning at PoC			Delays in transfer and isolation	Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical supplies			Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (175/1170, i.e. 15%)			high infection rates Nosocomial risks (119 cases of PPL, including 36 deaths)	Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility			Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Butembo, Mambasa and Logo)			Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Financial shortcomings	of the	resources	Limitation of the misery implemented in the pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)			Restriction of operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised			Increase in indirect (non-Ebola) mortality	Implementation and effectiveness of free healthcare, strengthening of CEHS

9 Photos of field activities



Deployment of laboratory inputs and equipment in the Mahagi health zone.



Joint mission to assess health facilities in Nizi (Health Zone, WHO, UNICEF)

**POUR TOUTE
INFORMATION
SUPPLÉMENTAIRE,
VEUILLEZ CONTACTER**

Pour l'Institut National de la Santé Publique (INSP) de la RDC

Le Directeur Général de l'INSP

Dr MWAMBA KAZADI Dieudonné

E-mail : dieudonne.mwamba@insp.cd

Tél. : +243 816 040 145

Le Coordonnateur COUSP

Prof NGANDU Christian

E-mail : Christian.ngandu@insp.cd

Tél. : +243998091915

L'Incident Manager

Prof Pierre AKILIMALI

E-mail : pierre.akilimali@insp.cd

Tél. : +243815800288

Avec l'appui technique de l'Organisation Mondiale de la Santé (OMS),
US CDC, Africa CDC et Blue square